

ROYAL FITNESS CLUB

RFC PREMIUM GUIDE #18

ADVANCED ANABOLIC CYCLE MASTERY

**Multi-Compound Protocols, Peptide Integration & Long-Term
Health**

5 Chapters · Advanced Level · Expert Knowledge · Health-First Approach

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DISCLAIMER

For educational and harm-reduction purposes only. Not medical advice. Consult a qualified healthcare professional before starting any protocol. Individual results vary.

CHAPTER 1 — ADVANCED CYCLE PHILOSOPHY

"Advanced users do not use more — they use smarter. Fewer compounds, better recovery, longer-term health management."

An advanced anabolic user is not defined by the number of compounds used simultaneously but by the depth of knowledge, disciplined monitoring, and long-term strategic thinking applied to every cycle. At advanced levels, the goal shifts from "maximum gains" to "optimal gains with minimum long-term cost." Health preservation becomes the primary KPI.

Advanced Cycle Characteristic	Description
Bloods every cycle	Full panel pre-cycle, mid-cycle, 4 weeks post-PCT — non-negotiable
Minimum compounds	Use the fewest compounds that achieve the goal — not the maximum tolerated
TRT base awareness	Consider whether TRT transition is better than repeated cycles
Cardiovascular monitoring	Echo + carotid IMT every 2–3 years at advanced use level
Systematic record keeping	Every compound, dose, date, side effect — 5-year log minimum
Exit strategy planned	Know when and how to transition off cycles permanently

CHAPTER 2 — MULTI-COMPOUND PROTOCOLS

Advanced Mass Stack (Experienced Users Only)

Compound	Dose	Ester	Duration	Role in Stack
Testosterone Enanthate	600–800mg/wk	Long	16 weeks	Primary anabolic base
Boldenone (EQ)	400–600mg/wk	Long	16 weeks	Appetite, RBC, lean mass
Anadrol (Oxymetholone)	50mg/day	Oral	Weeks 1–4 only	Rapid strength kickstart
Drostanolone (Masteron)	400mg/wk	Propionate	Last 4 weeks	Finishing hardness, anti-E2

Advanced Cutting Stack

Compound	Dose	Duration	Key Effect
Testosterone Propionate	100–150mg EOD	12 weeks	Base — minimal water retention
Trenbolone Acetate	75–100mg EOD	12 weeks	King of fat loss compounds
Masteron Propionate	100mg EOD	12 weeks	Hardening, natural AI
Anavar	50mg/day	Weeks 5–12	Strength preservation, dryness
Winstrol	25–50mg/day	Weeks 9–12	Final hardening effect

CHAPTER 3 — ADVANCED CUTTING COMPOUNDS

Trenbolone Deep Dive

"Trenbolone is uniquely powerful and uniquely demanding. It rewards discipline and punishes recklessness."

Trenbolone Acetate is a 19-nor compound with approximately 5× the anabolic and androgenic potency of testosterone. It does not aromatise into estrogen but elevates prolactin significantly. It causes direct fat oxidation, creates a pronounced hardening effect, and dramatically reduces appetite. Side effects are dose-dependent and include night sweats, insomnia, aggression, and cardiovascular strain.

Trenbolone Side Effect	Mechanism	Frequency	Management
Night sweats	Elevated metabolic rate, altered thermoregulation	Very common	Cool room, moisture-wicking sheets
Insomnia	CNS stimulation, cortisol effects	Common	Melatonin 1mg, no stimulants after 12pm
Aggression	Androgenic CNS effect	Common (dose-related)	Mindfulness, dose management, communication
Tren cough	Oil microemboli (propionate)		
Prolactin gyno	19-nor progestenic activity	Moderate risk	Cabergoline 0.25–0.5mg 2× weekly

CHAPTER 4 — PEPTIDE + STEROID INTEGRATION

Combining peptides with anabolic compounds creates a multi-axis hormonal enhancement that significantly exceeds the effect of either approach alone. The synergy is primarily at the GH-IGF-1 axis level.

Protocol	Compounds	Peptides	Goal	Duration
Advanced lean bulk	Test E 600mg + EQ 400mg	Ipamorelin + CJC + MK-677	Maximum lean mass	16 weeks
Advanced cut	Test P + Tren A + Mast P	GHRP-2 + CJC-1295 + T3	Maximum fat loss, preservation	12 weeks
Recomp	Test E 300–400mg (TRT+)	IGF-1 LR3 + Ipamorelin + MK-677	Simultaneous fat loss + muscle	16 weeks
Anti-ageing / maintenance	TRT dose only	Ipamorelin + CJC year-round (cycled)	Long-term health + body comp	Year-round

CHAPTER 5 — LONG-TERM HEALTH & TRT TRANSITION

"The most successful long-term enhanced athletes are those who manage their health with the same diligence they manage their training. Your future self is a stakeholder in your current choices."

Long-Term Cardiovascular Monitoring

Test	Frequency	Purpose	Action Threshold
Echocardiogram	Every 2–3 years	Left ventricular hypertrophy detection	LVH Stage 1 = dose reduction
Carotid IMT Ultrasound	Every 2–3 years	Atherosclerosis early detection	Any significant plaque = cycle reassessment
Calcium Score CT	Every 3–5 years	Coronary artery calcification	Score > 100 = aggressive lipid management
Lipid panel (advanced)	Every cycle	LDL particle size + ApoB	ApoB > 130 = intervention
Kidney function (eGFR)	Every 6 months	Creatine + compound metabolites	eGFR < 60 = nephrology referral

■ **TRT transition is a legitimate medical pathway for advanced users whose HPTA does not recover fully. A well-managed TRT protocol often presents fewer long-term risks than repeated supraphysiological cycles.**